ARES ADVANCE PRACTICE

Primary Care Provider and Transitional Care

DR. JEFF OLSEN, DO

NPI#1518301977

Patient: Delores, English

DOB: 8/29/1958

Date: 3/24/26

Home health Eval
for skilled nursing
PT Eval & treat

Signature:

DR. JEFF OLSEN, DO

Rx

Delores English

MRN :

Birthday : **1958-08-29**

Phone : **7027691862**

Visited on: 2026 Mar 19 11:00 (Age at visit: 67 years)

Electronically signed by: JEFFERY OLSEN, DO on 2026-03-24 11:33 AM

CC

Chief Complaint: Recent fall at home.

MEDICAL NECESSITY: This visit was a face-to-face encounter with the patient in her home. This is not a visit of convenience. The patient is seen in the home because there is a medical necessity for this visit to be conducted in the patient's environment. The patient suffers from progressive dementia, severe mobility limitations, and a recent fall, which are contraindications for the patient to be seen in an office setting today. It is medically necessary and safer for the patient to be seen in her own environment today.

Subjective

Delores D. is an elderly female with a history of progressive dementia, type 2 diabetes mellitus, atrial fibrillation, COPD, bipolar disorder, and nephrolithiasis who presents for routine follow-up care. The patient has multiple comorbidities and cognitive function that is progressively declining.

The patient recently completed a hospital stay and required 5 days of CNA assistance because she was unable to shower independently. She experienced one fall approximately 2 weeks ago when she missed the doorknob while trying to exit and fell to the floor. The patient has a significant smoking history of 30-40 years, previously averaging 2 packs per day, but reports she quit smoking one month ago and is currently using nicotine patches. She denies any current cigarette use.

The patient reports sleeping well and denies any current pain, including abdominal pain, chest pain, or arm pain. She states she feels good overall. The patient has hard of hearing and may be getting hearing aids pending approval. She lives with family support, with her daughter Rochelle providing care assistance. The patient denies alcohol use and reports no behavioral issues or wounds on her body.

Her last blood work was completed in September of the previous year. The patient is described as loving to eat and continues on diabetic medications but is not currently on insulin, though she was previously on insulin therapy.

Review of Systems:
- General: Reports persistent sleepiness and increased appetite.
- HEENT: Hard of hearing; no hearing aid in place.
- Cardiovascular: No chest pain reported.
- Respiratory: Known COPD; no acute dyspnea mentioned.
- GI: No abdominal pain.
- GU: No issues discussed.
- Musculoskeletal: Recent single fall; ambulation decline; no current pain.
- Skin: No wounds noted per patient/caregiver.
- Neuro/Psych: Progressive dementia; no behavioral disturbances; alert & oriented ×2

Medications

(Medication reconciliation last updated 04/20/2025 07:55 AM, JEE JEAN DYKHENG - Performed)
pioglitazone 30 mg tablet, Take 1 tablet by mouth once daily
digoxin 125 mcg (0.125 mg) tablet, Take 1 tablet by mouth daily
FIASP FLEX TOUCH PEN INJ, INJECT 15 UNITS SUBCUTANEOUSLY IN THE MORNING AND 15 UNITS IN THE EVENING AND 15 UNITS BEFORE BEDTIME
donepeziL 5 mg tablet, 1 tablet daily
Vitamin D3 125 mcg (5,000 unit) tablet, 1 tablet daily

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amoxicillin 500 mg tablet, 1 tablet twice a day for 5 days
Basaglar KwikPen U-100 Insulin 100 unit/mL (3 mL) subcutaneous, INJECT 35 UNITS SUBCUTANEOUSLY NIGHTLY
ELIQUIS 5MG TAB, TAKE 1 TABLET BY MOUTH IN THE MORNING, THEN 1 BEFORE BEDTIME
FIASP FLEX TOUCH PEN INJ, INJECT 15 UNITS SUBCUTANEOUSLY IN THE MORNING AND 15 UNITS IN THE EVENING AND 15 UNITS BEFORE BEDTIME
LOSARTAN 50MG TAB, TAKE 1 TABLET BY MOUTH IN THE MORNING FOR 90 DAYS
SPIRONOLACTONE 25MG TAB, TAKE 1 TABLET BY MOUTH IN THE MORNING
METOPROLOL TART 50MG TAB, TAKE 1 TABLET BY MOUTH TWICE DAILY (MORNING AND BEFORE BEDTIME)
METFORMIN 500MG TAB, TAKE 2 TABLETS BY MOUTH IN THE MORNING AND 2 IN THE EVENING WITH MEALS

Allergies

No known allergies

Mental/Functional

The patients speech was normal, sharing conversation with normal laryngeal efforts. Appropriate mood and affect were seen on exam. Thought processes were logical, relevant, and thoughts were completed normally. Thought content was normal. Thought content was normal with no psychotic or suicidal thoughts. The patients judgement was realistic with normal insight into their present condition. Mental status included: correct time, place, person orientation, normal recent and remote memory, normal attention span and concentration ability. Language skills included the ability to correctly name objects. Fund of knowledge included normal awareness of current and past events.
Reviewed HRA. Copy of HRA in documents section.

Functional Ability/Safety Screen
1. Does the patient need help with the phone, transportation, shopping, preparing meals, housework, laundry, medications or managing money? Yes
2. Does the patient's home have rugs in the hallway, lack grab bars in the bathroom, lack handrails on the stairs or have poor lighting? Yes
3. Has the patient noticed any hearing difficulties? No

Fall Screening: timed "Up and Go" test less than 30 seconds. Cognitive Test performed is MMSE and the result is Normal.

Vitals

Height/Length	Weight	BMI	Blood Pressure	Temperature	Pulse	RR	SpO2
5' 7"	**149 lbs 0 oz**	**23.34**	**141/63**		**75**	**16**	**97.0**

Recorded: 2026 Mar 21 22:03 by: GURJIT KAUR -

Objective

GEN: NAD
NECK: supple, NT, FROM
RESP: lungs clear to auscultation bilaterally, no rales, wheezes or rhonchi, nonlabored breathing, no use of accessory of muscles of respiration
CV: RRR, no m/r/g
GI: +BS, nontender to palpation, no masses, no HSM
DERM: skin warm and dry
EXT: no cyanosis/clubbing/edema
NEURO: AO x 3
PSYCH: judgment/insight intact, NL mood/affect

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Assessment

Malaise and fatigue (R53.81) Other malaise modified 21 Mar, 2026

Hearing loss (H91.90) Unspecified hearing loss, unspecified ear modified 21 Mar, 2026

Chronic obstructive pulmonary disease (J44.9) Chronic obstructive pulmonary disease, unspecified modified 21 Mar, 2026

Atrial fibrillation (I48.91) Unspecified atrial fibrillation modified 21 Mar, 2026

Muscle weakness (M62.81) Muscle weakness (generalized) modified 9 Sep, 2025

Diastolic heart failure (I50.32) Chronic diastolic (congestive) heart failure modified 11 Apr, 2025

Dementia (F03.90) Unspecified dementia without behavioral disturbance modified 11 Apr, 2025

Patient has inability to leave home and consequently leaving home would require a considerable and taxing effort

Plan

- Refer to home health services including PT, nursing and CNA care- Continue nicotine patch therapy for smoking cessation
- Monitor respiratory status
- Encouraged continued abstinence from smoking.
- Monitor respiratory status; no acute exacerbation today.
- Reassess need for pulmonary follow-up if symptoms change.

For worsening symptoms, call 911 or go to ER – Pt/CG verbalized understanding.

Follow-up: 4-6 weeks or PRN. Report acute changes.

- Financial, privacy, and consent to treat notices are signed and understood by patient/CG.

- It was explained to the patient/CG that the patient is permitted to have only one PCP or transitional care/gap provider at any given time. If the patient has more than one PCP or transitional care provider
at any given time, insurance may not cover and patient is responsible for the bill/coverage.

Falls: Risk for falls and fall prevention reviewed with Pt and family member. Falls can lead to severe injuries and morbidity that may lead to inpatient hospitalizations. Utilize assistive devices at all times,
decrease clutter, especially in the walkway, and rise slowly. Ask for help when needed.

Patients social history, medical history, family history, surgical history, and medication (current and past) was reviewed. Patient's current health status, prognosis, morbidity, and goals were carefully
considered. Patients plan of care was carefully and clinically considered and evaluated for the best outcome for the Pt.

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ACP- Discussed goals for medical treatment f2f for 30min. Patient comfortable following the provider's recommendation, which is guided by her explicit preferences. Advance care planning is a key
component of shared decision making and although it is not required to, may result in documentation of a living will, assignment of a health care proxy, or other advance directive.

CCM- Comprehensive personalized chronic care plan created, revised, and updated, as needed. Provided a comprehensive care plan to set and track progress towards health goals, and support between regular face- to-face visits. Spent 30min with patient.

Spent time 45min with Health behavior assessment including evaluation of the patient's responses to disease, illness or injury, outlook, coping strategies, motivation, and adherence to
medical treatment.
This encounter is medically necessary due to the change in the patients condition or chronic disease is progressing.
Pts assessment, evaluation and care plan is of moderate to high complexity.

Questions and concerns addressed with Pt/CG - verbalized understanding.

Malaise and fatigue
- Encourage energy conservation and rest
- Monitor for worsening symptoms or new onset of weakness
- Assess for reversible causes (e.g., anemia, infection, medication side effects) with labs as indicated
- Reinforce importance of adequate hydration and nutrition

Hearing loss
- Family to pursue hearing aid; awaiting insurance approval.

Chronic obstructive pulmonary disease
- Avoid exposure to respiratory irritants (smoke, dust, strong fumes)
- Take all medications as prescribed, including inhalers and nicotine patches
- Monitor for increased shortness of breath, cough, or sputum changes
- Practice breathing exercises as instructed by your provider or physical therapist
- Stay up to date with vaccinations (influenza, pneumococcal)
- Maintain adequate hydration and nutrition
- Contact your healthcare provider if you experience worsening symptoms or signs of infection

Atrial fibrillation
- Take all heart medications as prescribed (e.g., digoxin, metoprolol)
- Monitor for symptoms such as palpitations, dizziness, chest pain, or shortness of breath
- Maintain regular follow-up appointments and lab monitoring as directed
- Stay hydrated and avoid excessive caffeine or alcohol
- Notify your provider if you experience new or worsening symptoms

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- Understand the importance of medication adherence to reduce risk of stroke and complications

Muscle weakness

- Report any new or worsening muscle weakness to your provider promptly
- Engage in gentle physical activity or exercises as recommended by your provider or physical therapist
- Ensure adequate nutrition and hydration
- Avoid activities that increase fall risk if you feel weak
- Monitor for associated symptoms such as pain, numbness, or tingling
- Follow up with your healthcare team for ongoing assessment